

[FACILITY_NAME_001] [a government health facility] [LOCATION_002]

DEPARTMENT OF PEDIATRIC ONCOLOGY - [a specialty unit]

CASE SUMMARY: ADMISSION & CHEMOTHERAPY PROGRESS NOTES

REPORT ID: GMCH-[INCIDENT_REPORT_001]

DATE: [DATE_TIME_013]

[MEDICAL_RECORD_001]

Patient Name: [PERSON_009] [PERSON_006]

[PERSON_009] /Sex: [DATE_TIME_012] / Male

Aadhaar No: [AADHAAR_NUMBER_001]

MRN (Hospital ID): [a registration number]

Father's Name: [PERSON_008] [PERSON_006]

Mother's Name: [PERSON_005] [PERSON_006]

[PERSON_007] : [PERSON_008] [PERSON_006] [PHONE_NUMBER_001])

Address: House No. 224, Sector 15-C, [LOCATION_002] - [PINCODE_001]

ADMISSION NOTE ([DATE_TIME_011]):

[PERSON_009] [PERSON_006] presented with persistent low-grade fever and bone pain for [DATE_TIME_010] . [PERSON_003]

[PERSON_008] was previously seen at a local clinic where he was [PHI_DIAGNOSIS_002]

[PHI_DIAGNOSIS_002] . [PERSON_008] [PERSON_006] (father) reports that [PERSON_009] [PERSON_006] has been unusually lethargic. Upon examination, [PERSON_009] [PERSON_006] showed signs of pallor and mild hepatosplenomegaly.

[PERSON_008] [PERSON_006] mentions that there is no family [PHI_DIAGNOSIS_001] [PERSON_005] [PERSON_006]

(mother) was also interviewed; she noted that [PERSON_009] [PERSON_006] had a normal birth history.

[PERSON_009] [PERSON_006] blood work showed a high WBC count ([DATE_TIME_009]) and 65% blasts, suggestive of Acute Lymphoblastic Leukemia (ALL).

PROCEDURE NOTE ([DATE_TIME_008]):

Bone marrow aspiration and biopsy performed on [PERSON_009] [PERSON_006] under conscious sedation.

Procedure was explained to [PERSON_008] [PERSON_006] , and written informed consent was obtained

from [PERSON_008] [PERSON_006] in the presence of a witness. [PERSON_009] [PERSON_006] tolerated the procedure

well. [PERSON_008] [PERSON_006] remained by [PERSON_009] [PERSON_006] side during the recovery phase.

CHEMOTHERAPY LOG - PHASE I (INDUCTION):

[DATE_TIME_007] : Vincristine 1.5mg/m2 administered to [PERSON_009] [PERSON_006] . [PERSON_005] [PERSON_006]

showed no immediate hypersensitivity. [PERSON_008] [PERSON_006] was instructed on monitoring [PERSON_003]

[PERSON_008] for peripheral neuropathy symptoms.

[DATE_TIME_006] : L-Asparaginase 6000 IU/m2 given IM to [PERSON_009] [PERSON_006] . Nursing notes

indicate [PERSON_009] [PERSON_006] was cooperative. [PERSON_008] [PERSON_006] reported that [PERSON_009] [PERSON_006] had one episode of vomiting post-infusion.

NURSING HANDOVER:

[PERSON_009] [PERSON_006] is currently stable in [a specialty unit location] . [PERSON_009] [PERSON_006] vitals are being monitored every

[DATE_TIME_005] . [PERSON_008] [PERSON_006] is the designated bedside attendant. [PERSON_008] [PERSON_006] has been

provided with the pharmacy list for [PERSON_009] [PERSON_006] medications. [PERSON_005] [PERSON_006] will relieve

[PERSON_006] at [DATE_TIME_004] [DATE_TIME_003]. Please ensure [PERSON_003] MRN
([a registration number]) is checked before any medication delivery.

DISCHARGE PLAN (PROVISIONAL):

Once [PERSON_003] completes the induction phase, [PERSON_003] will be monitored as an outpatient. [PERSON_006] has been given a [DATE_TIME_002] helpline number. [PERSON_003] must return for follow-up on [DATE_TIME_001]. Dr. [PERSON_002], Senior Consultant, has reviewed [PERSON_003] progress.